

The plan

AGELESS, Wednesday 21 October 2026

FIFTEEN DAYS. CAPITAL CLUB PREPARED FOR SEVENTY GUESTS.
DR ITUNU AKINWARE AND DR CHINWE KPADUWA
CONSULT FOR AFRICA STATUS

The one job

Put seventy of the right people in a room at Capital Club on 21 October, and convert what happens there into a diary afterwards.

Everything else is subordinate to that. The evening is not the outcome: a full room that produces nothing on the 22nd is a party, and a half-empty one that produces fifteen consultations is a success.

Cutting from 130 to 70 helps more than it costs. Seventy is a room where people talk to each other and the clinical team can reach everybody, and it means every seat has to be worth something.

What the evening has to do

AGELESS is the platform and it is built to recur. The proposition is a new era of health, beauty and longevity. The panel is *The Ageless Blueprint*. There is no fireside. Keeping those names apart is what lets edition two reuse everything but the panel.

Medlyfe hosts, in the footer. That is a regulatory position rather than a courtesy: Medlyfe is the licensed, bookable entity. No practice is introduced, because the practice name that was being used does not exist. Dr Kpaduwa is named as a clinician and sets the clinical standard. Every piece of copy already survives that distinction and it should hold on the night.

What is already built and live

The event page is live at consultforafrica.com/lyfe, carrying the RSVP and a fallback discovery call, both writing into one queue. The coordinator queue at [/admin/lyfe-enquiries](https://admin/lyfe-enquiries) sorts by how long somebody has waited rather than when they arrived. Invitations are built in individual and group versions, personalised per guest, as print, feed and WhatsApp Status, including Dr Itunu's own with her name above the invitee. Alongside them sit six feed pieces, five Status pieces, a salon flyer, a hand-over card and a scope sheet for referring clinicians. The panel shortlist is a separate document, four candidates for each of six jobs, ready to mark up.

The page is deliberately kept out of search while the brand has no domain of its own. Links shared by hand still work.

Where seventy guests come from

Nobody is coming off an advertisement. Seventy is a hand-built list, and the only question is who owns each tranche.

Source	Roughly	Note
MedLYFE's patients and members	25	By telephone, from the consented book
Dr Itunu's and Dr Chinwe's own lists	15	Personally. A principal's forwarded invitation outperforms everything else here
A convening partner, most likely GAIA	12	Needs a decision this week. Their membership is this exact room
Referring clinicians	10	Guests, not a sales target. Next year's referral flow
Capital Club members	5	Ask the club. Their list overlaps ours
Press	3	Two interview slots in a side room

The mechanic that actually fills a room is a printed invitation delivered by hand against a telephone call already made. An email does not do it. The invitations are built to be personalised one per guest for exactly that reason.

The stage

Six jobs. One is filled.

Job	State
The panel seat on skin and aesthetics	Dr Chinwe Kpaduwa. Filled
Longevity and health optimisation	Four candidates. First call Dr Moyosore Makinde
Metabolic health and hormones	Four candidates. First call Dr Afokoghene Isiavwe
Performance and physical function	Four candidates. First call Ngozi Ojora
Skin and regenerative medicine	Four candidates. First call Dr Folakemi Cole-Adeife
Panel chair	Four candidates. First call Adesuwa Onyenokwe
Fireside host	Four candidates. First call Ezinne Akudo

Three calls matter more than the rest: the chair, because it decides whether the evening is a conversation or four presentations; the longevity seat, because it is what makes this an evening about ageing rather than about treatments; and the skin seat, because an academic there keeps the aesthetic half clinical.

Take the skin seat from the teaching hospitals, not from private clinics. Every private aesthetic dermatologist in Lagos sells what MedLYFE sells, and a rival with a microphone is worse than an empty chair.

The fifteen days

When	What	Owner
This week	Confirm the chair and four panel seats. Approach the convening partner. Capital Club walk-through, run of show, AV, press room. Invitations personalised, printed and hand delivered against calls already made	To assign
By Fri 9 Oct	Press approach, two preview interviews under embargo	To assign
By Fri 16 Oct	RSVP deadline. Every unanswered invitation chased by telephone, not email	To assign
Mon 19 to Tue 20	Final numbers, seating, run-through, brief the chair	To assign
Wed 21 Oct	The evening. Consultation desk staffed from the door, diary open	To assign
Thu 22 Oct	Every guest called inside 24 hours. The day it converts or does not	To assign
Week of 26 Oct	Consultation days at capacity. Every clinician who came, called personally	To assign

Two people have just joined the team and are not named against anything above. Tell me who they are and what they are covering and I will put names in this column, because an owner-less row is a row that does not happen.

What is blocking

Blocker	Why it matters
The MedLYFE logo file	Everything currently carries a rebuilt mark drawn from measured geometry off a flattened poster. It is close, it is not theirs. One file fixes every asset at once
RSVP name and direct line	Invitations and the page carry a placeholder number. Nobody can reply to a placeholder
Dr Kpaduwa's MDCN position	She can take a panel seat as a clinician speaking to her subject. She cannot be held out as available to treat patients until registration is settled, and the exposure sits on the clinic, not on her
Three CFA relationships on the panel list	Dr Chito Nwana, Dr Uju Rapu and Iwosan each need a decision from Debo before anyone telephones them
The convening partner	If GAIA or an equivalent is in, twelve seats fill themselves. If not, those twelve come off personal lists and the calling starts sooner
Two stage risks, both fixed by briefing the chair once	HEFAMAA has scope of practice live in Lagos, so a panel drifting into who may inject is a regulatory moment. And a clinician praising a named treatment from a stage you host is closer to an advertisement than it feels, with the ARCON liability on the advertiser. Discuss categories, not the menu

How we will know it worked

Measure	Target
Consultations booked on the night or within 48 hours	The number that matters. Everything else is attendance
Time from RSVP or enquiry to a human telephone call	Under an hour, every time. This is the single thing that killed the March campaign
Clinicians who attended and hold a direct line	Referral flow is a next-year asset, built this month
A named, consented list for edition two	The most durable output. AGELESS recurs, so the list and the format carry forward